

Slide 1 Reading Notes

National Hospital Network project charter | Meds Counselling LLC

Opening

Good morning everyone. We are Meds Counselling LLC, and this first slide introduces our project charter for National Hospital Network, or NHN. The slide summarizes the client background, the business problem, our team roles, key stakeholders, and the hypothesis we plan to test.

1. Overview of the business

NHN is a large integrated healthcare system with 42 hospitals and more than 300 outpatient sites across 11 states. It serves about 2.3 million patients each year, so even a modest readmission issue can become a systemwide clinical, operational, and financial challenge.

The scale table helps explain why this problem matters. A readmission pattern at this size can affect patient outcomes, bed capacity, care-team workload, and financial performance.

2. Problem Statement

The business problem is that 30-day readmissions continue to rise even though NHN has invested in discharge planning, care coordination, and follow-up services. The readmission rate increased from 14.2 percent in 2022 to 18.4 percent in 2025.

Leadership estimates more than 42 million dollars in annual readmission-related cost exposure. This means the project needs to explain why patients are coming back and where NHN can intervene in a practical and financially responsible way.

Transition cue

After explaining the problem, move to how the team is organized to investigate it.

3. Team roles by workstream

Our team is organized by workstream so that each part of the project has a clear owner. Can He leads the project story and presentation. Belal Glab owns data preparation and exploratory analysis. Jingyi Cao focuses on clinical risk analysis. Santosh Kumar Chantati leads predictive modeling. Jeevan-Kumar Mittapalli covers financial impact and ROI. Adarsh Pravinbhai Patel examines care coordination operations. Sam-Aquila Siddani supports dashboard design and recommendations.

Workstream	Owner
Project story and presentation	Can He
Data preparation and exploratory analysis	Belal Glab
Clinical risk analysis	Jingyi Cao
Predictive modeling	Santosh Kumar Chantati
Financial impact and ROI	Jeevan-Kumar Mittapalli
Care coordination operations	Adarsh Pravinbhai Patel

4. Identify Stakeholders

The key stakeholders include executive leadership and finance teams, clinical and operations teams, care coordinators and discharge planners, and patients, caregivers, and community partners. Each group needs a different answer from the project.

Leadership and finance need a clear view of cost exposure and return on investment. Clinical and operations teams need to know which patients are at highest risk and where the workflow breaks down. Care coordinators need to know which post-discharge actions matter most. Patients and caregivers need safer transitions after discharge.

5. Business Hypothesis

Our initial hypothesis is that readmissions rise when post-discharge support does not match patient risk. High-risk patients may need faster follow-up, stronger chronic disease support, medication review, transportation help, or coordinated outreach.

We will test which factors have the strongest relationship with readmission and which care coordination steps appear to reduce risk. If the data supports this hypothesis, NHN can focus resources on patients most likely to benefit from additional support.